

STEP THERAPY OVERRIDE · AUDIT WORKSHEET

Five Clauses Govern Step Therapy Overrides. *The Workflow Rarely Matches the Clause.*

Three audit passes. One override clause. One operational workflow.

This handout pairs with the Thursday LinkedIn post decoding the five step therapy override terms (trigger conditions, documentation burden, denial appeal timeline, prescriber attestation language, member cost-share treatment). Run the three audit passes when override approval rates fall below 60% — that is the threshold where the workflow is gating clinically appropriate therapy more than the contract intends.

01 Pull the override approval and denial rate

Open your most recent quarterly report. Find the prior authorization summary. Look for step-therapy-specific override numbers: total step therapy edits triggered, total override requests submitted, total approved, total denied.

If the report aggregates step therapy with general PA, request the breakout. Calculate the override approval rate.

02 Classify the denial reasons

For denied overrides, request the denial reason breakdown by category: insufficient documentation, prescriber attestation incomplete, clinical criteria not met, administrative timeout.

Category mix tells you what is causing denials. "Insufficient documentation" or "administrative timeout" dominance signals process failure, not clinical judgment.

03 Compare the override clause to the workflow

Open your contract's step therapy override clause. Note documentation burden, prescriber attestation requirements, denial appeal timeline.

Compare to the actual workflow: what does a prescriber submit, how long is the response window, who decides the appeal. Workflow more burdensome than the contract = clause vs. operations gap.

04 The Pattern PBS Sees Most Often

Across approximately 100 PBM contract reviews and audits annually at PBS: when override approval rates fall below 60%, the workflow is gating clinically appropriate therapy more than the contract or formulary intend. The override clause is the contract; the workflow is the operations; the gap is your audit.

05 Paste-Ready: Send to Your Broker or PBM

DATA REQUEST • TO YOUR PBM ACCOUNT TEAM

Subject: Step therapy override audit data – last four quarters

Please provide the following step-therapy-specific data, separately from general prior authorization:

1. Total step therapy edits triggered, by therapeutic class.
2. Total override requests submitted.
3. Total approved and total denied, with denial reason breakdown:
 - a. Insufficient documentation
 - b. Prescriber attestation incomplete
 - c. Clinical criteria not met
 - d. Administrative timeout
 - e. Other (specify).
4. Average response time from override request to determination.
5. Prescriber abandonment rate (overrides started but not completed).
6. Member therapy abandonment rate following step therapy denial.

This data is for internal workflow audit. Delivery requested within 14 business days.

06 This Quarter's Action Plan

STEP 1 • THIS WEEK

Pull the override approval/denial breakdown

Send the data request. The denial-reason category mix is the lead diagnostic.

STEP 2 • WEEK 2

Calculate override approval rate

If below 60%, surface for renewal. If denial reasons skew toward documentation/timeout, the issue is process not clinical.

STEP 3 • WEEK 3

Read the override clause against the workflow

Document specific gaps. Send written gap notification to the PBM.

STEP 4 • BEFORE NEXT QUARTERLY REVIEW

Bring the audit to your broker

Override workflow is a renewal-leverage item that compounds quarter over quarter.